

Product Requirements Document

Swasthya Katha — Visual Multilingual Health Magazine

Version: 1.0

Status: Draft for approval

Date: 24 August 2026

Owner: Product lead

Primary users: People with limited health literacy, community learners, and health workers

1. Executive summary

Swasthya Katha is a multilingual, visual health-learning platform designed to make important health information understandable, memorable, and actionable for people who may not be comfortable with long text or complex applications.

The product combines:

- Magazine-style digital storytelling.
- Simple linear learning.
- Accessible SVG illustrations.
- Optional WebGL visual enhancement.
- Audio narration.
- Word and image highlighting synchronized with narration.
- Local-language support.
- Offline-friendly reading.
- Privacy-preserving usage.
- Clinically reviewed content.

The first publication may address sexually transmitted infections and sexual health. The product must educate without diagnosing, prescribing, shaming, or exposing sensitive personal behaviour.

2. Problem statement

People often receive health information through dense text, unclear images, or clinical language. This creates several failures:

- Readers cannot identify the main message.
- Users may incorrectly assume that visible appearance confirms health status.
- People may not know what testing involves.

- Stigma can discourage care-seeking.
- Language and reading ability limit comprehension.
- Slow networks and low-end devices prevent consistent access.
- A visually impressive interface can still be inaccessible.

3. Product goal

Enable a reader to understand one important health message, remember it, and identify a safe next step in a short, respectful, multilingual experience.

4. Product non-goals

The first release will not:

- Diagnose users.
- Interpret personal clinical photographs.
- Recommend individualized medication.
- Replace a clinician or health worker.
- Host unmoderated user medical advice.
- Require an account for ordinary reading.
- Collect sexual history or diagnosis data.
- Treat automatic translation as medical approval.

5. Target audiences

Primary audience

- Readers with limited health literacy.
- Readers who prefer audio or visual explanation.
- People using low-cost Android phones.
- People with intermittent connectivity.
- People who prefer regional languages.

Secondary audience

- Community health workers.
- Peer educators.
- Teachers and facilitators.
- Clinicians and public-health reviewers.
- Translators and voice artists.
- Program managers.

6. Personas

Persona A — Independent learner

Needs a private, non-judgmental explanation. May have limited reading confidence and may share a device.

Success condition: Can start a lesson, understand its key message, and find the next safe action without assistance.

Persona B — Community health worker

Needs a reliable teaching aid with readable visuals, audio, captions, and discussion prompts.

Success condition: Can present a lesson to a small group and answer basic navigation questions without technical support.

Persona C — Clinical reviewer

Needs to inspect individual claims, sources, translations, visuals, and revision history.

Success condition: Can approve, reject, or request changes with a clear audit trail.

Persona D — Content editor

Needs a structured authoring process that prevents incomplete or unreviewed content from reaching production.

Success condition: Can create a lesson draft and submit it through clinical, language, accessibility, and product review.

7. Product principles

1. Comprehension before spectacle.
2. Dignity before shock value.
3. Anonymous access before unnecessary identity collection.
4. Human review before medical publication.
5. Linear fallback before advanced animation.
6. Local language and audio as first-class features.
7. Every meaningful visual has an alternative.
8. Every animation has a reason.
9. Every release is versioned and reversible.
10. Every completed feature has evidence.

8. Core user journeys

8.1 First-time reading

1. Open a public link.
2. Select language.
3. Select Read, Listen, or Simple mode.
4. Read a brief privacy explanation.
5. Open the first lesson.
6. Follow text and visual explanation.
7. Replay audio if desired.
8. Complete an optional comprehension check.
9. See a clear action and help pathway.

8.2 Returning reading

1. Reopen the application.
2. Restore local language and position.
3. Continue from the last lesson.
4. Change mode or accessibility settings.
5. Clear local history at any time.

8.3 Facilitated learning

1. Open facilitator mode.
2. Increase visual scale.
3. Select a lesson.
4. Display one visual step at a time.
5. Play narration and pause for discussion.
6. Use action and referral cards.

8.4 Content publication

1. Create draft.
2. Add approved claim sources.
3. Add plain-language adaptation.
4. Add visual storyboard.
5. Submit clinical review.
6. Translate after source approval.

7. Record narration.
8. Review accessibility and timing.
9. Approve release.
10. Publish version.

9. Functional requirements

9.1 Publication discovery

- Show publication title, purpose, language, version, and update date where appropriate.
- Display a clear start action.
- Provide privacy, accessibility, and help links.
- Avoid sensitive terms in browser notifications or unnecessary local metadata.

9.2 Reader modes

Magazine mode

- Two-page desktop spread.
- Single-page mobile view.
- Page-turn animation.
- Chapter navigation.
- Page counter.
- Visual storytelling layouts.

Linear mode

- Semantic vertical reading flow.
- Complete content without animation.
- Keyboard and screen-reader compatibility.
- Stable links to lesson blocks.
- Text and audio alternatives.

Facilitator mode

- Enlarged visuals.
- Presentation controls.
- Pause points.
- Discussion prompts.
- High-contrast option.

9.3 Narration

- Play, pause, stop, seek, and replay.
- Adjustable speed.
- Transcript.
- Word-level timing.
- Visual target timing.
- Language-specific audio.
- Graceful missing-audio fallback.

9.4 Visual explanation

- Semantic SVG scenes.
- Stable target IDs.
- Focusable visual targets.
- Dim and highlight states.
- Static fallback.
- Text alternative.
- Optional WebGL enhancement.

9.5 Localization

- Locale-based URLs.
- Language selector.
- Translated navigation and errors.
- Translation status tracking.
- Language-specific audio.
- Translation-aware layout testing.

9.6 Progress and bookmarks

- Local progress without login.
- Optional account-based synchronization later.
- Bookmark lesson or chapter.
- Clear all local state.
- Do not store sensitive topic history by default.

9.7 Content operations

- Draft creation.
- Revision history.
- Source register.
- Role-based review.
- Publication gate.
- Release version.
- Rollback.
- Scheduled re-review.

10. Non-functional requirements

Accessibility

Target WCAG 2.2 AA as the minimum engineering baseline. All essential content must be available without WebGL, colour-only cues, or audio.

Performance

- Core content must appear before optional effects.
- Heavy visuals must be lazy-loaded.
- Lite mode must be available.
- Audio must not block reading.
- Low-end Android testing is mandatory.

Privacy

- Anonymous reading.
- Minimal analytics.
- No health status collection.
- Local data clearing.
- Clear privacy notice.

Security

- Staff MFA.
- Server-side authorization.
- Row-level security.
- Sanitized SVG.
- Audit trail.

- Secrets management.

Reliability

- Linear fallback for reader failures.
- Static fallback for visual failures.
- Transcript fallback for audio failures.
- Versioned content.
- Rollback support.

11. Initial publication structure

1. Welcome and privacy.
2. What is an STI?
3. How can it spread?
4. Some infections have no visible signs.
5. Prevention and protection.
6. Testing.
7. Treatment and professional care.
8. Partner communication.
9. Myths and facts.
10. Help and referral information.

The final clinical scope, wording, visuals, and referral information require country-specific review. WHO provides an authoritative starting point for STI prevention, diagnosis, and treatment content. [web:27]

12. Success metrics

Comprehension

- At least 80% of pilot users explain the key message correctly.
- At least 80% identify the appropriate safe next action.
- Less than 10% interpret a visual as a diagnosis tool.

Usability

- At least 90% change language without help.
- At least 90% find the testing/help section without help.
- At least 90% can replay narration.

Accessibility

- 100% of core information available in linear mode.
- No critical keyboard or screen-reader blocker.
- Reduced-motion mode removes nonessential motion.

Engineering

- No P0/P1 release defects.
- Core lesson works on defined low-end device set.
- Rollback completes within the agreed operational target.

These are proposed targets. They become official only after product-owner approval and baseline measurement.

13. Acceptance criteria for M0

M0 is complete when:

- Audience is confirmed.
- Launch geography is confirmed.
- Initial languages are confirmed.
- Clinical reviewers are assigned.
- Three prototype lessons are selected.
- Security and privacy model is approved.
- Reader modes are approved.
- Initial backlog is prioritized.
- Evidence plan is approved.

14. Open decisions

- First launch country and service referral network.
- Age range.
- First languages.
- Account requirement, if any.
- Offline scope.
- Clinical review organization.
- Safeguarding process for minors.
- Hosting and data-residency requirements.